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A Comprehensive Course

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MODULE THREE: TAKING CARE OF BABIES

KEY LEARNING OBJECTIVES

- **Crying and comforting.**
- **Holding a young baby.**
- **Nappy changing routine.**
- **Making up formula milk.**
- **Making up formula routine.**
- **Winding the baby.**
- **Spoon feeding.**
- **Breathing precautions and problems.**
- **Sleeping.**
- **Caring for Baby skin.**



1. FEEDING

Adequate nutrition during infancy and childhood is critical to child health and development. Globally under nutrition is a leading cause of childhood mortality. The 2005 Innocent Declaration on Infant and Young Child Feeding recognizes that inappropriate feeding practices; sub-optimal or no breastfeeding and inadequate complementary feeding, remain the greatest threat to child health and survival.

Early under nutrition and micronutrient deficiencies have been associated with impairment of intellectual performance, work capacity, and overall health and nutritional status during adolescence and adulthood. In addition to under-nutrition the rising incidences of overweight and obesity in children are also a serious concern.

In several countries the dilemma over infant feeding in the context of HIV, especially in developing countries where breastfeeding is the cultural norm, has posed a new threat to optimal infant nutrition, child health, survival and development.

Studies have shown that inadequate support for infant and young child feeding is the main contributing factor to inappropriate feeding practices globally. There is a need for health care personnel to receive up to date evidence based knowledge and skills on appropriate infant and young child feeding practices to provide quality counselling and adequate support to mothers and caregivers.

Children need good nutrition to grow and develop, and they are ready for new foods as their digestive systems mature. Children's eating experiences are influenced by variables such as the physical and social settings, the expectations of the adults, the children's physical abilities, and their sensitivities to taste, texture and temperature. Early experiences associated with eating and feeding may influence children's future attitudes and practices around food. Families, caregivers and health professionals can work together to ensure that children are well-nourished and develop healthy eating habits.

1.0 **Hygienic Preparation of Food.**

A baby who is not breastfed is at increased risk of illness for two reasons:

- Replacement feeds may be contaminated with organisms that can cause infection.
- The baby lacks the protection provided by the breast milk.

After six months of age all children require complementary feeds. Clean, safe preparation and feeding of complementary foods are essential to reduce the risk of contamination and the illnesses that it causes. The main points to remember for clean and safe preparation of feeds are; *Clean hands, Clean utensils, Safe water and food, Safe storage.*

1.1.1 **Clean Hands.**

Always wash your hands;

- after using the toilet
- after cleaning the baby's bottom
- after disposing of children's stools, and after washing nappies and soiled cloths.
- after handling foods which may be contaminated, for example, raw meat and poultry products.
- after touching animals.
- before preparing or serving food.
- before eating, and before feeding children.

However, it is not necessary to wash hands before every breastfeed if there is no other reason to wash them. It is important to wash your hands thoroughly. with soap or ash. with plenty of clean running or poured water. front, back, between the fingers and under the nails. Let your hands dry in the air or dry them with a clean cloth. It is best not to dry them on your clothing or a shared towel.

1.1.2 **Clean Utensils.**

- You need to keep both the utensils that you use, and the surface on which you prepare feeds, as clean as possible.
- Use a clean table or mat, that you can clean each time you use it.

- Wash utensils with cold water immediately after use to remove milk before it dries on, and then wash with hot water and soap.
- If you can, use a soft brush to reach all the corners.
- Keep utensils covered to keep off insects and dust until you use them.
- Use a clean spoon to feed a baby complementary foods. Use a clean cup to give a baby milk or fluids.
- If a caregiver wants to put some of the baby's food into her mouth to check the taste or temperature, she should use a different spoon from the baby.

1.1.3 Safe Water and Food.

- Safe water and food are especially important for babies.
- Bring the water to a rolling boil before use. This will kill most harmful micro-organisms. A rolling boil is when the surface of the water is moving vigorously. It only has to 'roll' for a second or two.
- Put the boiled water in a clean, covered, container, and allow to cool.
- The best kind of container has a narrow top, and a tap through which the water comes out.
- This prevents people from dipping cups and hands into the water, which can make it not safe.
- If the water has been stored for more than 48 hours it is better to use it for something else, for example cooking or give to older children to drink.
- Fresh cow's milk or other animal's milk to be used for a baby also needs to be briefly boiled to kill harmful bacteria.
- Boiling also makes the milk more digestible. The milk and water can be boiled together.
- Milk sold in the shops may already have been heat-treated in various ways such as pasteurization, UHT (ultra-high temperature) or sterilization. These treatments kill the harmful micro-organisms, and they help the milk to keep longer if it is not opened.
- It can be used without boiling if it is used immediately on opening. After it is open, it will only keep as long as fresh milk.
- If it has been open more than an hour, it will need to be boiled before giving it to a baby.

- Some families keep water cool in a pottery jar, which allows evaporation of water from the surface. This method is not safe for milk.
- If a mother is giving complementary foods, she should prepare them freshly each time she feeds the baby, especially if they are semi-liquid.

1.1.4 Safe Storage.

- Food should be kept tightly covered to stop insects and dirt getting into it.
- Food can be kept longer when it is in a dry form, such as milk powder, sugar, bread and biscuits, than when it is in liquid or semi-liquid form.
- Fresh fruits and vegetables keep for several days if they are covered, especially if they have thick peel, like bananas.
- Fresh milk can keep in a clean, covered, container at room temperature for a few hours. Exactly how long depends on the condition of the milk when bought, and what the room temperature is.
- However, for an infant, milk must be boiled and then used within an hour of boiling.
- If the house does not have a refrigerator, make feeds freshly each time. When a feed has been prepared with formula or dried milk, it should be used within an hour, like fresh milk.
- If a baby does not finish the feed, you should give it to an older child or use it in cooking.
- Some families keep water hot in a thermos flask. This is safe for water. But it is not safe to keep warm milk or formula in a thermos flask.
- Bacteria grow when milk is kept warm.
- Discuss with the mother or other caregiver how the household routine works – whether the mother cooks once or twice a day, whether she can prepare feeds many times a day, how often she goes to the market and what facilities she has for storage. Help her to find ways of preparing the baby's food in a clean and safe way.

1.2 **Bottle Feeding.**

If the baby is only bottle-feeding, you should obviously start right after birth. If the baby is being breastfed, however, it's recommended that you wait about three weeks until introducing a bottle. Bottle-feeding earlier could potentially interfere with the successful establishment of breastfeeding, not because of “nipple confusion” (which is debatable), but because breasts may not be stimulated enough to pump up supply. If you wait much later, though, baby may reject the unfamiliar bottle in favor of the breast because that's what she's gotten used to.



1.2.1 **Check the expiration date**

Look for an expiration or "use by" date on the formula container. You can't be sure of the formula's quality after that date. Don't buy or use expired infant formula.

1.2.2 **Wash your hands**

Before preparing formula, wash your hands thoroughly with soap and water. Dry your hands well. Clean the area where you will prepare the formula.

1.2.3 Prepare your bottle

Sterilize bottles, nipples, caps and rings before using them for the first time. You can sterilize them with any of these methods:

- Boil the bottle and accessories in water for five minutes.
- Use a microwave steam sterilizer bag.
- Use a stand-alone electric steam sterilizer.

After the first use, you usually don't need to sterilize bottles and their parts. Wash these items with soap and hot water. Bottle and nipple brushes can help you clean them well. You also can use a dishwasher.

You might continue sanitizing the bottle if your baby:

- Is younger than 3 months.
- Was born prematurely.
- Has a weakened immune system.

1.2.4 Add water to concentrated liquid or powdered formula

Formula comes in three forms:

- Ready-to-feed liquid.
- Concentrated liquid.
- Powder.

Do not add water to ready-to-feed liquid formula. You will need to add water to concentrated liquid and powder formulas. Follow the instructions on the product label.

Water safety

You can use clean tap water or bottled water for concentrated liquid or powder formula. Other safety tips are:

- Ask your public water supplier to test for lead in your water. If you have concerns, talk to your health care provider about lead testing for your child.

- If you use well water, have the water tested for chemicals, germs or minerals that may be harmful. Your local health department may provide a testing service.
- If you use well water or are concerned about water safety, boil the water for one minute. Measure the water after boiling and let it cool to room temperature.

Fluoride

Most public water supplies have fluoride added. Fluoride is important to prevent tooth decay. But too much fluoride can increase the risk of faint white lines or streaks on the teeth, called fluorosis.

If your baby drinks only concentrated or powder formula, you might switch between fluoridated tap water and low-fluoride bottled water. Labels on these bottles will say:

- Deionized.
- Purified.
- Demineralized.
- Distilled.

If your baby uses only ready-to-feed formula or you only use low-fluoride bottled water for formula, talk to your health care provider. After age 6 months, your baby may need fluoride supplements. This might be done by using fluoridated bottled water called nursery water. Or you may need to give your baby fluoride drops.

1.2.5 Measure the formula

Carefully measure the amount of water and formula. Too much water might mean your baby doesn't get enough nutrients. Too little water might cause your baby to have digestive problems or not have enough fluids.

For ready-to-use formula:

- Pour enough formula for one feeding into a clean bottle.
- Use only formula — don't add water or any other liquid.
- Attach the nipple and cap to the bottle.

For liquid-concentrate formula:

- Pour the amount of water needed into a clean bottle.
- Pour the amount of formula needed into the bottle.
- Attach the nipple and cap to the bottle and shake well.

For powdered formula:

- Determine the amount of formula you want to prepare, following instructions on the package.
- Measure the amount of water needed and add it to a clean bottle.
- Use the scoop that came with the formula container to scoop the powdered formula. Add the number of scoops needed into the bottle.
- Attach the nipple and cap to the bottle and shake well.

If your baby is younger than 3 months, was born prematurely or has a weakened immune system, take these additional steps. These steps can reduce the risk of illness from *Cronobacter* bacteria:

- Boil the water for one minute.
- Measure the water after boiling.
- Let cool for five minutes.
- Prepare the formula.
- Put the bottle under cold running water or in an ice bath to cool. The formula should be at body temperature, 98.6 degrees Fahrenheit (37 degrees Celsius).
- Test that the formula is lukewarm — not hot — by putting drops on your wrist or the back of your hand.

1.2.6 Warm the formula, if needed

It's fine to give your baby room temperature or even cold formula. If your baby prefers warm formula, place a filled bottle in a bowl of warm water or run under warm water.

Test the temperature by putting a couple of drops on the back of your hand or wrist. The formula should feel lukewarm — not hot.

Don't warm bottles in the microwave. The formula might heat unevenly, creating hot spots that could burn your baby's mouth.

1.2.7 Know when to throw or store

Use the formula within one hour after the beginning of feeding. Throw away formula left in the bottle and do not store it in the refrigerator. Bacteria from your baby's saliva can grow in the formula.

If you're using ready-to-use formula, cover and refrigerate any unused formula from a freshly opened container. Discard any leftover ready-to-use formula that's been in the refrigerator more than 48 hours.

If you prepare and fill several bottles of liquid-concentrate or powdered formula at once, do the following:

- Label each bottle with the date and time that the formula was prepared.
- Refrigerate the extra bottles until you need them.
- Discard any prepared formula that's been in the refrigerator more than 24 hours.

If you're not sure whether a container or bottle of formula is safe, throw it out.

1.3 REFLUX PRECAUTIONS

Reflux is when the muscle just above your baby's stomach isn't yet strong enough to keep the milk from running back up into the esophagus or food tube. Babies fed mother's milk are less likely to have reflux. The following may help:

- When nursing, keep your baby's head higher than his stomach. The cradle and football holds are the easiest positions for keeping your baby more upright.
- Hold your baby upright with the head higher than his/ her stomach after feedings.
- Change your baby's diaper before a feeding.
- More frequent burping may help if your baby spits up after feedings.
- If your baby goes home on medication for reflux, give the medication until your baby's doctor tells you to stop.

- Place a blanket under one end of the baby's mattress, so his/her head will be higher than his/her bottom when the baby is sleeping only when told to do so by the baby's doctor.

Vomiting

It is normal for your baby to have a wet burp or to spit up some formula after or during feedings. It is not normal to vomit before, during, or after feedings. If you are unsure if your baby is spitting up or vomiting, contact your health care professional.

1.4 Spoon Feeding.



1.4.1 The Role of Early Spoon-Feeding

Spoon-feeding the baby purees plays a specific, useful role. Babies develop better lip control and movement as they suck a puree off a spoon. It also limits the amount of food the baby will spit out and gets more food into your baby's tummy. However, most parents place the spoon in their child's mouth and then scrape the food off on the top of their baby's lip as they remove the spoon. Instead, you can teach the baby to suck the food off the spoon.

1.4.2 How to Start Spoon Feeding Baby

Step #1: Make sure that they're 6 months old, have good head control, and can sit upright to eat and swallow.

Step #2: Get them in the right position.



The position of the baby is often overlooked. It's easy to sit them on your lap while feeding them or feed them while they're in the car seat. But that isn't the safest position and can impair the development of their feeding skills.

Sitting on your lap or not in a supported seat is actually more challenging for babies. If the baby is focusing all their energy on sitting upright, they aren't able to use their mouth as effectively. To ensure the baby is seated properly, position them in an age appropriate booster seat or highchair with a footrest.

Ideally their hips, knees and feet should be at a 90-degree angle if possible. This means that when babies are first learning to eat, they should have support under their feet and they shouldn't

look slumped over. Not every highchair or booster seat does a good job keeping your little one in the right position.

Step #3: Use a spoon with a flat small bowl, it's easier for the baby to remove the food.

The bowl of the spoon is so important. You want it to be mostly flat or to have a shallow bowl. That means it can't hold too much of the puree on it. The spoon should also have a narrow bowl. If the baby is right around 6 months old, their mouth isn't big. The narrow bowl fits in their mouth much better than a big wide spoon.

Step #4: Choose a totally smooth thinner baby food.

If you're using store bought baby food use stage 1 for the first feedings. If you're making homemade baby food, you don't want the food to be too thick. Think about making it thick enough for baby to swallow, but not so thin it's like liquid. Homemade baby food at this stage should spill off the spoon easily, but still have some stuck to the spoon when you turn it upside down.

Step #5: Place the right amount of puree on the spoon.

You'll want to avoid overfilling or under-filling the spoon. You should have some puree on it, ideally near the front of the spoon. Avoid scooping and loading up the spoon with as much puree as possible. This can be really overwhelming to your baby, especially if they're just learning to eat purees.

Too much food in their mouth can also trigger their gag reflex. If the baby does gag, don't panic, it's normal. Gagging and choking are two different things.

Step #6: Let the baby decide when to take a bite.

Place the spoon right in front of your baby and wait until they open their mouth. This puts them in charge of their responsibility right from the start, they are the ones that decide to eat.

You want to avoid opening their mouth for them or trying to sneak a bite in their mouth when they look away.

Helping the baby learn that they get to decide when to take a bite can reduce feeding battles and picky eating in the years to come.

Step #7: Place the spoon in the center of their mouth.

The spoon should go in the center of their mouth, above their tongue. You may gently use the back of the spoon to add a small bit of pressure to the middle belly of the tongue. This is helpful if their tongue seems to be moving quite a bit.

Step 8: Wait for them to close their mouth.

When you're excited and you really want them to take a bite, it can be hard to wait! But be sure to wait until they close their mouth on the spoon.

This lets them know what their job is, and helps lay the foundation for them to be able to feed themselves.

1.4.3 What to Avoid When Spoon Feeding Baby...

You have 8 simple steps to follow! But, there are a few things you'll want to avoid. You'll likely see others doing these things while feeding their babies, because they're all very common.

However, each of them can cause difficulties with eating and swallowing either in the moment you're feeding your baby or in the future. Here's what to avoid...

- **Avoid scraping food on the roof of your baby's mouth.** You want to wait until your little one closes their mouth on the spoon.
- **Don't feed them in a reclined position.** Car seats, strollers, and infant positioners all should be avoided. An upright high chair or booster seat is ideal to make sure your baby doesn't gag, choke, or aspirate.
- **Avoid distractions like the TV, phone, or toys.** If they're distracted while eating, they aren't fully experiencing all the different flavors and textures.
- **No force feeding.** Let your little one decide when to take a bite. This reduces the chance of picky eating later on and sets them up for positive mealtimes in the future.
- **Don't overfill the spoon.** You want a medium amount of puree on the spoon, near the front of the bowl.

1.4.4 Troubleshooting Spoon Feeding Challenges

It's very normal for babies to need some help getting the hang of eating purees. They're learning and may need a little more time. Here's some common challenges you might be faced with and what you can do...

Challenge #1: The baby won't open their mouth

When the baby won't open their mouth for the spoon, this can be really discouraging. Modeling is especially helpful in these cases. Take your own spoon and a little bite of the puree. Open your mouth wide and really over exaggerate what you're doing.

Show the baby how to do it. You can also try placing a mirror in front of the baby while they eat or give them their own spoon that is pre-dipped with puree to try out.

Challenge #2: The baby won't close their mouth on the spoon

If the baby won't close their mouth on the spoon, scraping the puree into their mouth is tempting. You want to avoid this!

It doesn't teach them how to use their muscles to eat. It also teaches them that they have to eat a particular food, which can encourage pickiness later on.

Frequently, if they don't close their mouth on the spoon, it's because they don't know they're supposed to. You can encourage this by very gently rolling your finger above their upper lip in a downward motion towards their mouth. Showing them how to take a bite with an exaggerated close is also helpful.

Challenge #3: The baby gags with purees

Gagging can get a bad wrap. Gagging happens first as a protective mechanism to prevent the baby from choking. But it can also turn into a problem if it starts to happen often. Or, before food even hits their mouth.

Sometimes excessive gagging happens because the baby's mouth is sensitive to sensory input, or the textures of food. It can be helpful to desensitize their mouth with teethingers or tooth brushing. You can brush their gums even if they don't have teeth yet!

Challenge #4: The baby throws their bowl or spoon

We hear parents tell us that their baby is throwing their food, plate, or utensils all the time. If the baby is doing this, you're not alone at all! Usually this happens just because they're testing things out at first. It makes a fun noise or you make a funny face whenever they do it.

It can be tempting to react negatively whenever food is thrown. Try to resist the urge and stay as neutral as possible. A negative reaction is still a reaction and can sometimes encourage them to do it more.

Try using a simple phrase like, "our food stays on the tray" and move on. It can also be helpful to use plates and bowls that suction well to their highchair to buy you a bit more time before the food flies.

Challenge #5: The baby always wants to hold the spoon

This is actually such a good thing, even though it can feel frustrating when you're just introducing purees. They're motivated to feed themselves!

Give them a spoon of their own that has been dipped in a little bit of puree while also having your own spoon. You can take turns doing the feeding. It might be frustrating because they will make a big mess at first and likely will get just a little bit of puree in their mouth. This is a great step towards their independence!